

CJC-1295 with DAC

GHRH Analogue | Long-Acting (DAC) | 2mg & 5mg x 10 Vials

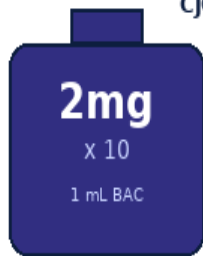
Once-Weekly Injection | ~7-Day Half-Life | Sustained GH Elevation

Purity: 99.8% | Endotoxin-Free | Research Use Only

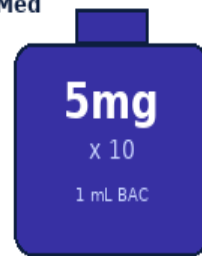
CJC-1295 with DAC (Drug Affinity Complex) is a long-acting GHRH (Growth Hormone Releasing Hormone) analogue. The DAC modification covalently binds CJC-1295 to serum albumin, extending its half-life from ~30 minutes (without DAC) to approximately 6-8 days. This allows once-weekly dosing while maintaining sustained elevation of GH baseline levels. Unlike CJC-1295 without DAC, the DAC version does not require fasted injection.

DAC ADVANTAGE: Once-weekly injection maintains a continuous elevated GH baseline all week. Stack with a daily GHS (Ipamorelin, GHRP-2/6) to trigger acute pulses on top of this elevated baseline for maximum amplification.

CJC-1295 DAC Vial Size Comparison -- SaiyanMed



2mg
2 mg/mL
2,000 mcg/mL

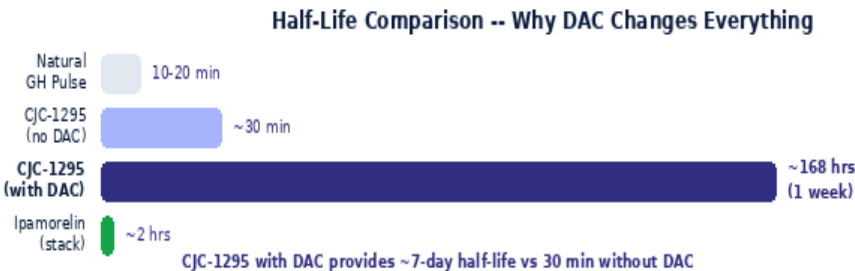


5mg
5 mg/mL
5,000 mcg/mL

Parameter	Specification
Vial Sizes	2mg x 10 vials 5mg x 10 vials Purity: 99.8%, Endotoxin-Free
Reconstitution	1 mL BAC Water per vial (both sizes) 2mg -> 2 mg/mL 5mg -> 5 mg/mL
Half-Life (DAC)	~6-8 days (168+ hours) -- due to covalent albumin binding via DAC
Standard Dose	1-2 mg (1,000-2,000 mcg) per week Start at 1mg/week
Frequency	Once weekly SubQ -- same day each week. No fasting required.
Fasting Required?	NO -- CJC-1295 DAC works through albumin binding, not GH pulse mechanism. Inject any time.
Stack With	Ipamorelin, GHRP-2, GHRP-6 (daily GHS) -- triggers pulses on elevated baseline
Cycle	12-16 weeks on, 4-6 weeks off Can run longer with monitoring
Storage	2-8 deg C unreconstituted Reconstituted: 2-8 deg C, use within 28 days

MECHANISM OF ACTION & RESEARCH BENEFITS

CJC-1295 with DAC binds to GHRH receptors (GHRHR) on pituitary somatotroph cells, stimulating sustained GH synthesis and release. The DAC (Lys17-GRF) modification forms a covalent thioester bond with Cys34 on serum albumin, dramatically extending circulating half-life. The result is a continuous, sustained elevation of baseline GH levels throughout the week, which stacks synergistically with acute GH pulses triggered by a daily GHS.



Pathway	Mechanism	Research Benefit
GHRHR Activation	Binds GHRH receptor on pituitary somatotrophs; activates adenylate cyclase via Gs protein; raises cAMP; stimulates GH gene transcription and GH granule exocytosis	Sustained GH release; elevated GH baseline throughout the week with once-weekly dosing
DAC Albumin Binding	Drug Affinity Complex (Lys17 modification) forms reversible covalent bond with Cys34 on serum albumin (t1/2 ~19 days); slowly releases active CJC-1295 over 6-8 days; protects from DPP-IV cleavage	Once-weekly dosing; stable plasma levels; no peak-trough fluctuations of short-acting GHRH
IGF-1 Elevation	Sustained GH elevation stimulates hepatic IGF-1 production; IGF-1 rises gradually over 4-8 weeks of consistent use; mediates anabolic effects	Progressive anabolic effect; lean mass accretion; fat loss; recovery enhancement
GHS Synergy (Stacked)	Elevated GHRH baseline primes pituitary somatotrophs and increases GH store; daily Ipamorelin triggers acute release from this enlarged pool; GHRH + GHS = 3-10x GH pulse vs either alone	Maximum GH output when stacked; peak body composition research results

Key Research Benefits

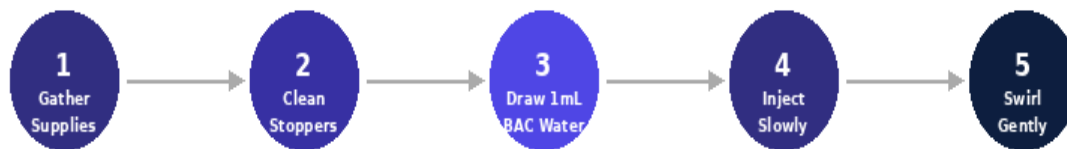
Benefit	Evidence & Research Context
Once-Weekly Convenience	DAC modification enables a single weekly injection maintaining sustained GH elevation throughout the week -- no daily injections, no fasting required.
Elevated GH Baseline	Sustained GHRH receptor stimulation raises average GH and IGF-1 levels progressively over the cycle, producing cumulative body composition improvements.
GHS Synergy	Adding a daily GHS (Ipamorelin) on top of the CJC-1295 DAC baseline produces acutely amplified GH pulses -- the most effective GHRH-based protocol.
No Fasting Requirement	Unlike CJC-1295 without DAC or GHS peptides, the DAC version can be injected any time regardless of food intake, improving protocol compliance.

CJC-1295 with DAC vs without DAC -- Key Differences

Feature	CJC-1295 WITHOUT DAC	CJC-1295 WITH DAC (This Product)
Half-life	~30 minutes	~6-8 days (168+ hours)
Injection frequency	2-3x daily	Once weekly
Fasting required?	YES -- always fasted	NO -- any time
GH effect	Acute pulsatile -- mimics natural GHRH	Sustained baseline elevation -- gradual and continuous
Typical dose	100-300 mcg per injection	1-2 mg once weekly
Best use	Stack with GHS same injection	Weekly base; stack daily GHS on top
Cycle	12 weeks on / 4 off	12-16 weeks on / 4-6 off

Reconstitution Protocol

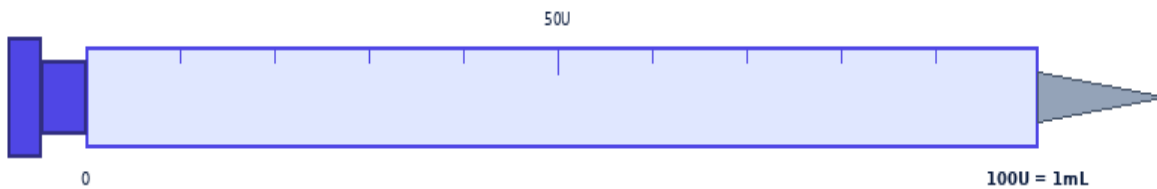
Both vial sizes (2mg and 5mg) use 1 mL BAC Water. Allow both the vial and BAC Water to reach room temperature (~20 deg C). Never shake -- roll gently. Solution should be clear and colourless.



Both vial sizes use 1 mL BAC Water. Swirl gently -- never shake.

Step	Action	Detail
1	Gather supplies	CJC-1295 DAC vial, BAC Water vial, insulin syringe (29-31G), alcohol swabs, sharps container, date label.
2	Clean stoppers	Wipe rubber stoppers of BOTH vials with a fresh alcohol swab. Air-dry 10-15 seconds.
3	Draw 1 mL BAC Water	Draw exactly 1 mL BAC Water. Remove all air bubbles. Use for both 2mg and 5mg vials.
4	Inject into CJC vial	Direct stream against the GLASS WALL -- never onto the powder. Inject slowly.
5	Swirl gently	Roll between palms 20-30 seconds. NEVER shake. Solution should be clear within 60 seconds.
6	Label and refrigerate	Write reconstitution date. Store at 2-8 deg C. Use within 28 days. Never freeze.

DOSE CALCULATION GUIDE



Insulin Syringe -- 1mL / 100U | 29-31G needle | SubQ only

Formula: Volume (mL) = Desired Dose (mcg) / Concentration (mcg/mL) | **Units:** Volume (mL) x 100 = Units on syringe

Vial: 2mg x 10 -- Add 1 mL BAC Water → Concentration: 2.0 mg/mL (2,000 mcg/mL)

Dose (mcg)	500 mcg	750 mcg	1,000 mcg	1,500 mcg	2,000 mcg
Volume (mL)	0.2500	0.3750	0.5000	0.7500	1.0000
Syringe Units	25.0U	37.5U	50.0U	75.0U	100.0U
Doses / Vial	4	2	2	1	1

Vial: 5mg x 10 -- Add 1 mL BAC Water → Concentration: 5.0 mg/mL (5,000 mcg/mL)

Dose (mcg)	500 mcg	750 mcg	1,000 mcg	1,500 mcg	2,000 mcg
Volume (mL)	0.1000	0.1500	0.2000	0.3000	0.4000
Syringe Units	10.0U	15.0U	20.0U	30.0U	40.0U
Doses / Vial	10	6	5	3	2

Tip: The 5mg vial yields 5 doses of 1mg (1,000mcg) or 2-3 doses of 2mg (2,000mcg). For 1mg/week protocol: each 5mg vial lasts 5 weeks. For 2mg/week protocol: each 5mg vial lasts 2-3 weeks.

INJECTION TECHNIQUE, SITES & WEEKLY SCHEDULE

No fasting required for CJC-1295 with DAC. Inject on the same day each week at any time -- morning, afternoon, or evening. SubQ injection into abdominal fat is preferred. Rotate sites each week to prevent localised tissue changes.

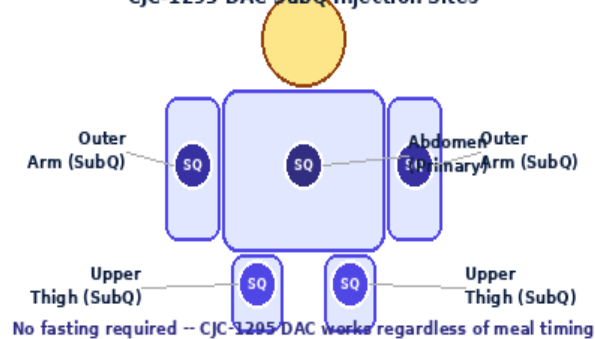
Once-Weekly Protocol -- Same Day Every Week



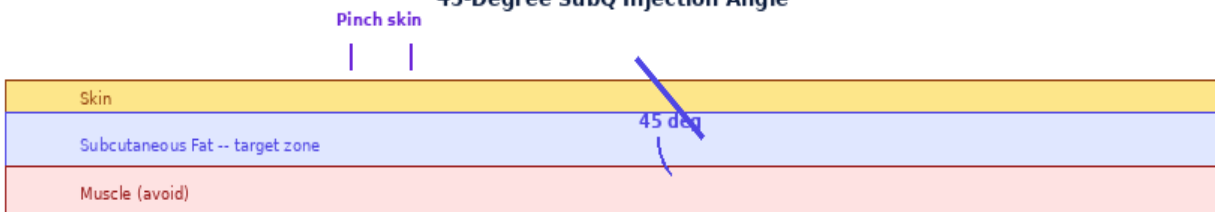
Choose any day -- stay consistent. 1-2 day variance is acceptable.

Step	Action	Detail
1	Wash hands	Wash with soap and warm water 20 seconds. Dry completely.
2	Select syringe	29-31G insulin syringe (1 mL). For doses under 500 mcg, a 0.3 mL syringe provides better precision.
3	Draw dose	Draw exact volume per dose table. Expel all air bubbles.
4	Clean site	Wipe site with alcohol swab. Allow 10-15 seconds to dry.
5	Pinch and inject	Pinch 2-3 cm skin. Insert at 45 degrees. Release pinch. Inject slowly over 5-10 seconds.
6	Withdraw, press, rotate & dispose	Remove at same angle. Press with dry swab 5-10 sec. Do NOT rub. Note site used. Rotate next week. Needle into sharps container.

CJC-1295 DAC SubQ Injection Sites



45-Degree SubQ Injection Angle



GHRH + GHS Synergy Stack

CJC-1295 DAC + GHS Synergy Stack



Stack GHS (Ipamorelin) on top of elevated GHRH baseline for amplified pulses

Stack Partner	Class	Dose	Frequency	Effect
Ipamorelin	GHS (selective)	100-300 mcg	1-3x daily fasted	Best stack: clean pulses on elevated baseline; no cortisol or prolactin
GHRP-2	GHS (non-selective)	100-300 mcg	2-3x daily fasted	Strong GH release; slight cortisol/prolactin spike; more aggressive protocol
GHRP-6	GHS (non-selective)	100-300 mcg	2-3x daily fasted	Strong GH; hunger side effect; similar to GHRP-2 profile

Research Dosing Protocols

Protocol	Weekly Dose	Frequency	Duration	Notes
Beginner	1 mg (1,000 mcg)	1x weekly	12 weeks on / 4-6 weeks off	Start here. Assess tolerance. Can inject any time, no fasting.
Standard	1-2 mg (1,000-2,000 mcg)	1x weekly	12-16 weeks on / 4-6 weeks off	Most common research dose. Stack with daily Ipamorelin.
Optimised Stack	2mg CJC-DAC (weekly) + 200mcg Ipa (daily)	CJC: 1x/wk Ipa: 1-2x/day	12-16 weeks on / 4-6 off	Maximum GH output. CJC-DAC raises baseline; Ipamorelin triggers acute pulses on top.

Side Effects

Side Effect	Likelihood	Management
Water retention / oedema	Common	GH-mediated. More common at 2mg/week. Reduce dose if significant. Usually resolves after 1-2 weeks.
Tingling / numbness (hands, feet)	Common	GH effect on nerve tissue. Typically mild and transient.
Joint discomfort	Occasional	Fluid in joint spaces. Usually resolves. Reduce dose if persistent.
Fatigue (initial weeks)	Occasional	Transient adaptation phase. Usually resolves after 2-3 weeks.
Injection site reaction	Occasional	Normal local reaction. Rotate sites weekly.
Headache	Occasional	Common in first 1-2 weeks. Ensure hydration. Usually self-resolving.

Contraindications: Active cancer or malignancy history (GH/IGF-1 stimulation), pregnancy or lactation. Not for use under 18. Monitor for acromegaly symptoms with prolonged high-dose use.

Storage Guidelines

State	Temperature	Duration	Notes
Unreconstituted powder	2-8 deg C	Per expiry	Upright. Away from light. Do not freeze.
Reconstituted solution	2-8 deg C	28 days	Label with reconstitution date. Inspect before each use.
BAC Water (opened)	2-8 deg C	28 days	Label with opening date.
Transport	Below 25 deg C	Max 72 hr	Insulated bag with ice packs. No direct ice contact.

Golden Rules -- CJC-1295 with DAC

1. Once weekly injection only -- the DAC half-life is ~7 days; more frequent dosing adds no benefit.
2. No fasting required -- inject CJC-1295 DAC any time of day regardless of meals.
3. Both vial sizes (2mg and 5mg) use exactly 1 mL BAC Water for reconstitution.
4. Never shake the vial -- roll gently between palms for 20-30 seconds until clear.
5. Direct BAC Water against the glass wall, never onto the peptide powder.
6. Stack with a daily GHS (Ipamorelin) to trigger acute GH pulses on the elevated baseline.
7. When stacking: the GHS must still be injected fasted -- the CJC-DAC dose does not.
8. Rotate injection sites each week. Record which site was used for systematic rotation.
9. Store reconstituted at 2-8 deg C. Label with date. Discard after 28 days.
10. Contraindicated in any cancer history. Take mandatory 4-6 week off-cycle breaks.

DISCLAIMER: This document is produced by for informational and research reference purposes only. CJC-1295 with DAC is a research peptide not approved for human therapeutic use. This guide does not constitute medical advice. Consult a qualified medical professional before beginning any peptide research protocol. assumes no liability for misuse of this product or information herein.

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